INSTITUTE OF LIVER & BILIARY SCIENCES

(An Autonomous Society under Government of NCT of Delhi)

D-1, Vasant Kunj, New Delhi, India

Phone No: 011-46300000 **Ext:** 17055 & 17056, 011-26706700– 02

Liver help line - 1-800-11-5354

Fax No: 46300010, **Email:** info@ilbs.in, **Website:** http://www.ilbs.in

DISCHARGE SUMMARY (HEPATOLOGY)

Patient Name: Mr. RAJEEV KAKKAR

DOB/Age: 04 Feb 1962/64Y 1M **Gender**: MALE

Mobile Number: 9810267452 **Insurance Card No**: 123

Address: . House Number-1577A, Pocket-1, Sector-B, Vasant Kunj, Vasant Vihar, South West Delhi, Delhi Delhi ,INDIA ,110070

Entitlement: IFFCO-TOKIO TPA

UHID: ILBS.0000413324

Case Number: IP/ILBS/26/06760

Ward / Bed: GENERAL WARD D/1047-03

Consultant:Dr. RAKHI MAIWALL

Department: HEPATOLOGY

Admission Date: 01 Mar 2026, 12:41

Discharge Date: 14 Mar 2026; 11:51

Payer: IFFCO-TOKIO TPA

ADMIT TYPE: EMERGENCY

DISCHARGE TYPE: NORMAL

DIAGNOSIS

- Other Acute Pancreatitis- Comments : Sequale of acute pancreatitis – Month 2 of illness- Etiology Microlith vs Hypercalcemia Local complications – WON (A predominantly organized Peripancreatic collection: Body – total span 9 X 4 cm- not increasing in size). Acalculus cholecystitis, no features of GOO. Systemic complications - None CTSI 4/10, CRP: 80 □ 3.5 CO-MORBIDITIES: Aortic valve sclerosis | Renal stones | Pulmonary arterial hypertension| Obesity | Fatty liver| Psoriasis |

Type	Code
Primary	K85.8

CHIEF COMPLAINT

ABDOMINAL PAIN X 1 DAY FEVER X 2 DAYS

ADMITTED FOR

Evaluation and the management of the symptoms

CURRENT ISSUES:

1. Acute Necrotizing Pancreatitis - A predominantly organized Peripancreatic collection: WON measures 9 x 4.5 cm with internal contents of 30-45 HU likely soft tissue debris- No ascites.
2. Hypercalcemia under evaluation, PTH: Normal, Vit D3: Normal, USG Parathyroid: Normal FDG PET: no abnormal uptake, except pancreatic cysts, Bone marrow: reactive marrow and cultures are negative. Familial hypercalcemic hypocalcuria also under consideration, UPEP, SPEP: awaited
3. (24H Urinary creatinine clearance – 0.28 | 24H urinary calcium – 0.06) – Single dose of **zolendronate given 13.2.26.**

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4.

HISTORY

Mr. Rajeev Kakkar, a 64-year-old male, with known h/o Aortic valve sclerosis | Renal stones | Pulmonary arterial hypertension| Obesity | Fatty liver| no h/o any addiction, had his index presentation in the form of sudden abdominal pain on 18/12/25. He was evaluated via MRI and was found to have acute pancreatitis with a complication of a pseudocyst (4.2 x 5 x cm and 35 x 31 x mm). A repeat CT on 26/02/26 revealed a large necrotic walled-off collection measuring 12 x 8 x 5cm with a 6 mm thick wall involving almost the entire body and tail of the pancreas, was admitted to ILBS CECT s/o - 10-20% of size reduction, was managed conservatively and discharged. He also gives h/o groin fungal infection. He has now come to ILBS with the above-mentioned complaints of abdominal pain and obstipation. There is no h/o jaundice, cough, altered bowel habits, hematemesis, and melena, burning micturition, altered sensorium or decreased urine output. There is no h/o any intoxications, indigenous medications, major surgeries, blood transfusions or IV drug abuse prior to the onset of the disease. There is no h/o DM/TB/COPD/Thyroid disorders.

GENERAL PHYSICAL EXAMINATION

Pt. was conscious, oriented to time place and person.

RR: - 20/min, SPO2: - 98% ON RA, Temp: - 98.4 BP: - 112/80 mm Hg, Pulse: - 97/min.

Pallor-, Icterus-, Cyanosis-, Clubbing-, Pedal edema (pitting type) - , LNP-, JVP normal

On systemic examination

Respiratory system:--B/L vesicular breathing, B/L airway equal air entry, no wheeze, no crepts

Cardiovascular system: - S1 S2 normal, no murmurs

CNS: - Conscious and oriented with no sensorimotor deficit,

Per Abdomen examination

ON INSPECTION: - non distended, umbilicus central and inverted, skin over the abdomen is stretched with no visible venous prominences, no visible pulsations.

ON PALPATION: - Soft, non-tender, liver non- palpable. Spleen non- palpable. No guarding, no rigidity, tenderness + Right hypochondrium.

ON PERCUSSION: - Tympanic note, no free fluid present.

ON AUSCULTATION: Normal bowel sounds present.

SYSTEMATIC REVIEW

Patient was admitted with above mentioned complaints. His examination findings at admission are mentioned above. Patient was started on conservative management with IV antibiotics, IV fluids and other supportive measures along with nutrition. His initial and latest lab data - Hb/TLC/PLT - 12.4/7.63/241, PT/INR - 13/1.09, BU/S.Creat - 16.2/0.78, with serum $Na^+/K^+/Cl^-$ - 136/6.01/100.1, Ca^{2+}/Mg^{2+}- 10.7/2. LFT showed S.Bil - (Total/Direct/Indirect: 3.18/1.44/1.74, AST/ALT - 407.4/291.7, SAP/GGTP - ,391.5/305.7 Alb/Glob - 3.61/3.6. URM - normal study. Sepsis screening was done is which showed (Blood c/s No bacterial growth

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after 48 hours of incubation). Po4 – 2.9, Amylase – 82.7, Lipase – 38.3, Vitamin D3 - 36, PTH - 58, CRP – 88 -- > 3.6 & PCT – 0.13 ACE - 36.□ PROTEIN ELECTROPHORESIS – awaited.□**IR Screening** was done – s/o A predominantly organized peripancreatic collection: 4 x 3 cm in tail of pancreas. 2 x 2.5 cm in body of pancreas. No ascites. The collection size was not increasing on serial images. On abdominal imaging he had features of acalculus cholecystitis. **PET CT** showed, features of acute pancreatitis with walled off necrosis.□Few peripancreatic lymph nodes with mild metabolic activity-likely reactive.□Distended gallbladder with thickened and oedematous walls showing increased metabolic activity -features of acute cholecystitis.□Mild and diffuse metabolic activity in the visualised axial and proximal appendicular skeletal marrow-likely reactive changes.□No evidence of abnormal metabolically active lesions elsewhere. **NCCT WA –** FUC of pancreatic WON. On the present study, the WON measures 9 x 4.5 cm **CT calcium score is 700.** CT Coronary Angio was planned but was not done. MRI WA + MRCP were planned. Patient and attendants were explained in great details regarding the nature and course of disease and prognosis. Proper diet, purging and mobilization was followed throughout the course of admission. All vitals, temperature, RBS and other necessary parameters were checked regularly and managed appropriately. In view of advanced age, Hypercalcemia, PTH normal, evidence of calcification elsewhere (arteries), endocrinology opinion is being taken regarding further evaluation and management options. Currently patient is hemodynamically stable and is being discharged on advised to follow up in OPD.

ADVICE

CONTINUE SOFT DIET

INJ MEROPENEM 1 GM IV TDS

INJ CLEXANE 0.6 MCG SC BD

INJ CALCITONIN 100 MG SC BD (FOR HYPERCALCEMIA) (discussed with Endocrinologist)

TAB PANTOP 40 MG PO OD

TAB CREON 40KIU PO TDS

TAB UDCA 450 MG PO BD

TAB GANTON 50 MG PO TDS

PEPTAMINE POWDERR 2 SCOOPS PO TD

TAB ROZUVASTATION 10 MG PO HS

TAB ECOSPRIN 75 MG PO BD

TO DO MRI WA + MRCP AND CT CORONARY ANGIOGRAPHY AND TO DO PTHrP REVIEW WITH□PROTEIN ELECTROPHORESIS REPORTS IN OPD

REVIEW IN HEPATOLOGY OPD WITH REPORTS OF CBC/LFT/KFT/INR/CRP **ON MONDAY.**

TO REVIEW WITH CARDIOLOGY CENTRE **FOR CT CALCIUM SCORE – 700.**

PLAN – APAXIBAN TO START ON OPD BASIS.

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WHEN TO OBTAIN URGENT CARE

Vomiting of blood / blood in stool or black colour stool.Any alteration in state of consciousnessSevere Dizziness or fainting spellsFever, new onset or worsening of abdominal painWorsening of jaundice Chest pain / shortness of breathSevere nausea, vomiting.

SENIOR RESIDENT

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TREATING FACULTY

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Discharge Summary Received at: 12:20 PM

Bill ready at 13.36 pm